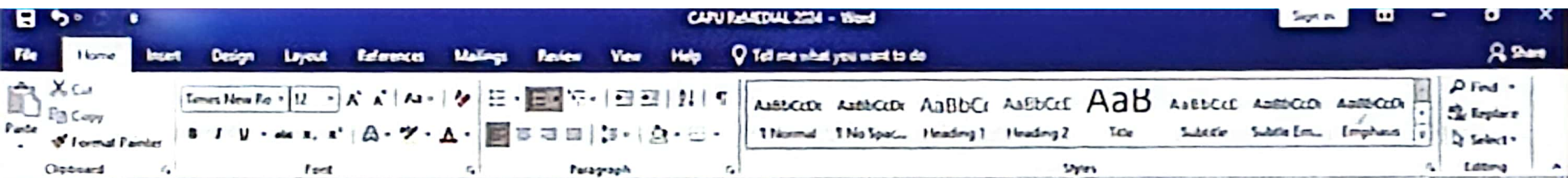


- 1) Correct about EXTERNAL intercostal –
 - A) Extends from the lateral border of the sternum to angle of the rib
 - B) Posterior to the angle it becomes continuous with the internal intercostal membrane.
 - C) Fibers directed downwards, backwards and laterally
 - D) It is **inspiratory muscle**
- 2) Pain originating from inflammation of parietal pleura referred to the abdomen as abdominal pain and misdiagnosed as acute abdomen. Which nerve/s are responsible for referring the pain to the abdomen?
 - A) T1 and T2
 - B) T4 to T6
 - C) **T7 to T11**
 - D) T3 to T6
- 3) Anterior intercostal arteries of in the first seven anterior intercostal spaces originate from
 - A) pericardiophrenic artery
 - B) descending thoracic aorta
 - C) Superior epigastric artery
 - D) **Internal thoracic artery**

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- B) descending thoracic aorta
 - C) Superior epigastric artery
 - D) **Internal thoracic artery**
- 4) During plural fluid aspiration or needle thoracostomy which site of the intercostal space is the safest site to pass the needle and tap the pleural fluid?
- A) Close to the inferior border the rib above
 - B) **Close to the superior border of the rib below**
 - C) In the most upper part of the ICS
 - D) Any point of the ICS
- 5) Which of the following layers provides a natural cleavage plane for surgical separation of the costal pleura from the thoracic wall?
- A) Deep fascia
 - B) **Endothoracic fascia**
 - C) Parietal pleura
 - D) Visceral pleura
 - E) Transversus thoracis muscle fascia
- 6) The apex of the heart is formed by which heart chamber?
- A) Left atrium
 - B) **Left ventricle**
 - C) Right atrium
 - D) ~~Right ventricle~~

- 7) Pericardial cavity is between
- A) Fibrous pericardium and serous pericardium
 - B) Visceral and parietal layers serous pericardium
 - C) Fibrous pericardium and parietal layers serous pericardium
 - D) Fibrous pericardium and visceral layer of serous pericardium
- 8) Which smaller cardiac veins drain directly to the Right atrium?
- A) middle cardiac vein
 - B) Great cardiac vein
 - C) Anterior cardiac veins
 - D) Coronary sinus
- 9) The following anatomic events occur at the level of the sternal angle (angle of Louis) except:
- A) The right and left pulmonary arteries enter the lungs
 - B) The trachea bifurcates
 - C) The ascending aorta becomes continuous with the arch of the aorta
 - D) The second costal cartilages articulate with the sternum
 - E) The right recurrent laryngeal nerve arises from the right vagus nerve
- 10) Which abnormal shape of chest may be due emphysema?
- A) Barrell chest
 - B) Pectus carinatum
 - C) Pectus excavatum



14. The most abundant cells in the respiratory epithelium are:

- A. Basal cells
- B. Brush cells
- C. Small granule cells
- D. Ciliated columnar cells
- E. Goblet cells

15. Which one of the following is the most external cardiac structure?

- A. Mitral valve
- B. Fibrous pericardium
- C. Purkinje cells
- D. Fibrous skeleton
- E. Epicardium

16. All of the following are true about large elastic artery except?

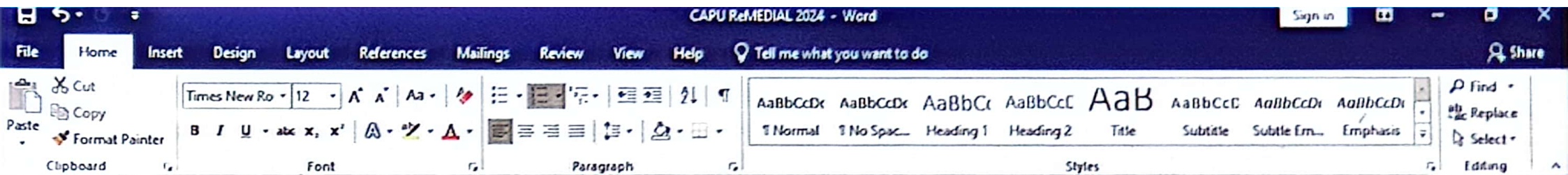
- A. The tunica media is rich in elastic fibers

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E. Epicardium

16. All of the following are true about large elastic artery except?

- A. The tunica media is rich in elastic fibers
- B. Tunica media is thicker than tunica adventitia
- C. Lined by endothelium
- D. The tunica adventitia contains vasa vasorum
- E. The internal elastic lamina is visible

17. Which one of the following bypasses directly shunts the blood from right atrium to left atrium?

- A. Ductus arteriosus
- B. Ductus venosus
- C. Foramen ovalae
- D. Umblical vein
- E. Umblical artery

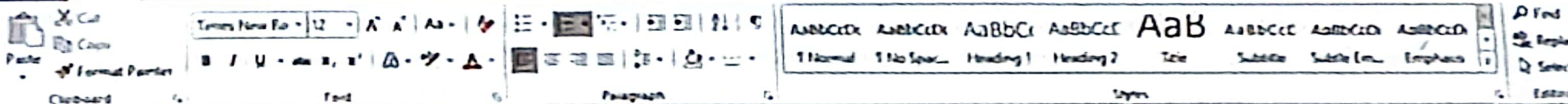
18. In fetal circulation, mixing of oxygenated and deoxygenated blood doesn't occur in which one of the following?

- A. Liver sinusoids

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C. Foramen ovalae

D. Umblical vein

E. Umblical artery

18. In fetal circulation, mixing of oxygenated and deoxygenated blood doesn't occur in which one of the following?

A. Liver sinusoids

B. Left ventricles

C. Inferior vena cava

D. Right atrium

E. Left atrium

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19. Which one of the following doesn't belong to the common components of Tetralogy of Fallot?

- A. Pulmonary stenosis
- B. Ventricular septal defect
- C. Dextrocardia
- D. Dextroposition of the aorta
- E. Right ventricular hypertrophy

20. During which maturational stage of the lung that the lung tissue appears an exocrine gland?

- a. Terminal sac stage
- b. Pseudoglandular stage
- c. Canalicular stage
- d. Alveolar stage
- e. Glandular stage

21. The QRS wave of the ECG represents depolarization of:

- a. ventricular myocardium
- b. atrial muscle

b. ~~Canalicular stage~~

c. Canalicular stage

d. Alveolar stage

e. Glandular stage

21. The QRS wave of the ECG represents depolarization of:

a. ventricular myocardium

b. atrial muscle

c. base of the ventricles

d. SA node

22. Closure of the aortic valve occurs at the onset of which phase of the cardiac cycle?

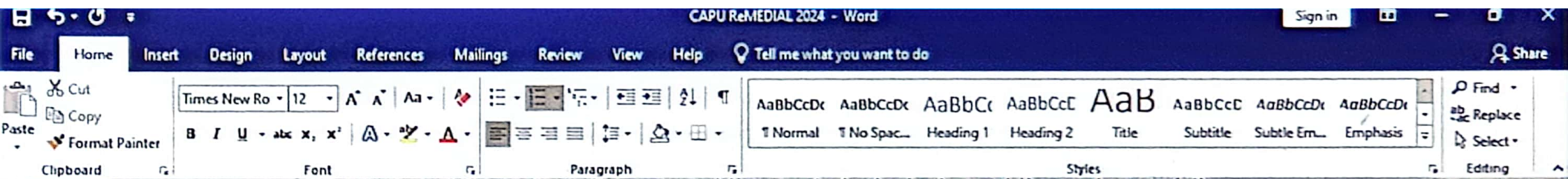
a. isovolumetric contraction

b. rapid ejection

c. isovolumetric relaxation

d. atrial systole

e. rapid ventricular filling



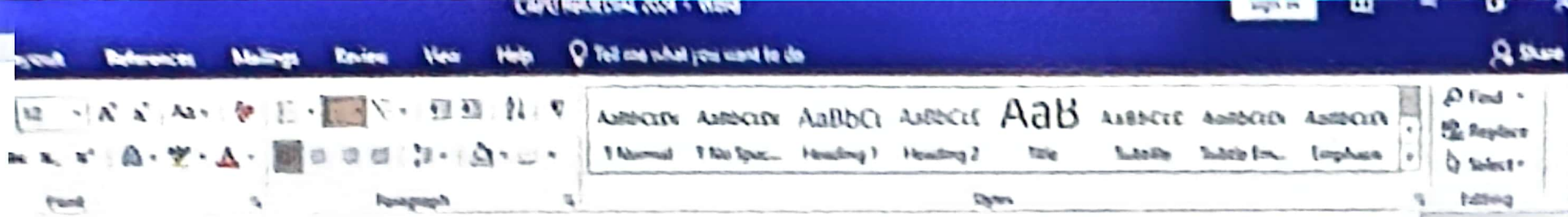
22. Closure of the aortic valve occurs at the onset of which phase of the cardiac cycle?

- a. isovolumetric contraction
- b. rapid ejection
- c. isovolumetric relaxation
- d. atrial systole
- e. rapid ventricular filling

23. The second heart sound is produced by:

- a. closure of the mitral valve
- b. opening of semilunar valves
- c. opening of AV valves
- d. closure of semilunar valve
- e. closure of the aortic valve

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24. Which of the following conditions would increase the afterload placed on the ventricle?

- a. Hypertension
- b. Aortic valve stenosis
- c. Increased total peripheral resistance
- d. Emboli
- e. All of the above

25. An individual with a blood pressure of 140/90:

- a. has a pulse pressure of 50 mm Hg
- b. has a mean arterial pressure of 140 mm Hg
- c. systolic pressure has not been determined
- d. has a diastolic pressure of 140 mm Hg
- e. has a heart rate of 100 beats per minute

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e. has a heart rate of 100 beats per minute

26. All of the following are true concerning mean arterial pressure EXCEPT:

- a. It is the average pressure in the aorta over time
- b. It is dependent on arterial compliance
- c. It is directly proportional to cardiac output
- d. It would be increased with an increase in stroke volume but not change in heart rate or TPR
- e. It is directly proportional to total peripheral resistance

27. A -66 year old man experiences orthostatic hypotension following sympathectomy. The explanation to this occurrence is:

- a. An exaggerated response of the renin-angiotensin –aldosterone system (RAAS)
- b. A suppressed response of the RAAS system
- c. An exaggerated response of the baroreceptor reflex mechanism
- d. A suppressed response of the baroreceptor reflex response
- e. An exaggerated shift of blood flow to the kidneys

28. Which of the following factors does not increase the contractile state of the heart?

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- b. It is dependent on arterial compliance
- c. It is directly proportional to cardiac output
- d. It would be increased with an increase in stroke volume but not change in heart rate or TPR
- e. It is directly proportional to total peripheral resistance

27. A 66 year old man experiences orthostatic hypotension following sympathectomy. The explanation to this occurrence is:

- a. An exaggerated response of the renin-angiotensin –aldosterone system (RAAS)
- b. A suppressed response of the RAAS system
- c. An exaggerated response of the baroreceptor reflex mechanism
- d. A suppressed response of the baroreceptor reflex response
- e. An exaggerated shift of blood flow to the kidneys

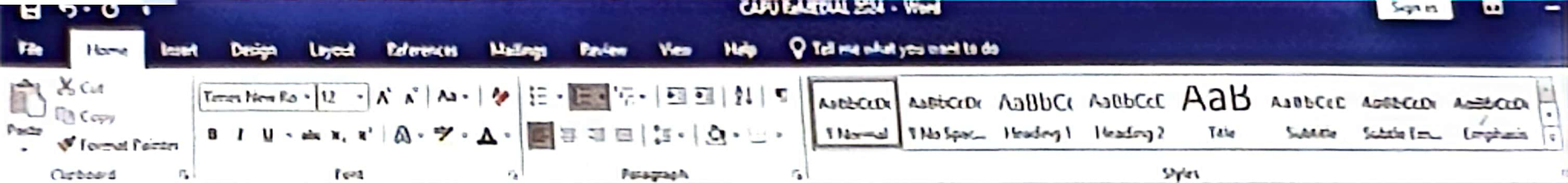
28. Which of the following factors does not increase the contractile state of the heart?

- a. β 1-sympathetic stimulation
- b. Increased extracellular calcium concentration
- c. Increased extracellular sodium concentration
- d. Cardiac glycosides

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29. Why is the myocardium of the right ventricle thinner than that of the left ventricle?
- A. the RV pumps into the pulmonary circuit which has less resistance than the systemic circuit
 - B. the RV pumps a smaller volume of blood than the LV
 - C. the RV pumps blood out with a slower exit speed than the LV
 - D. the RV chamber has a smaller volume than the LV
30. Which of the following events occur during isovolumetric relaxation?
- A. the atria are relaxed, the ventricles are filling passively, and the atrioventricular valves are open
 - B. the ventricles are starting to contract, the atrioventricular valves are closed, the semilunar valves are open
 - C. the atria contract, the ventricles are relaxed, the atrioventricular valves are open
 - D. the atria are relaxed, the ventricles are starting to relax, the atrioventricular valves are closed, the semilunar valves are closed.

2.1 What would be a possible consequence of the CA node failing to deauthorize?



valves are open

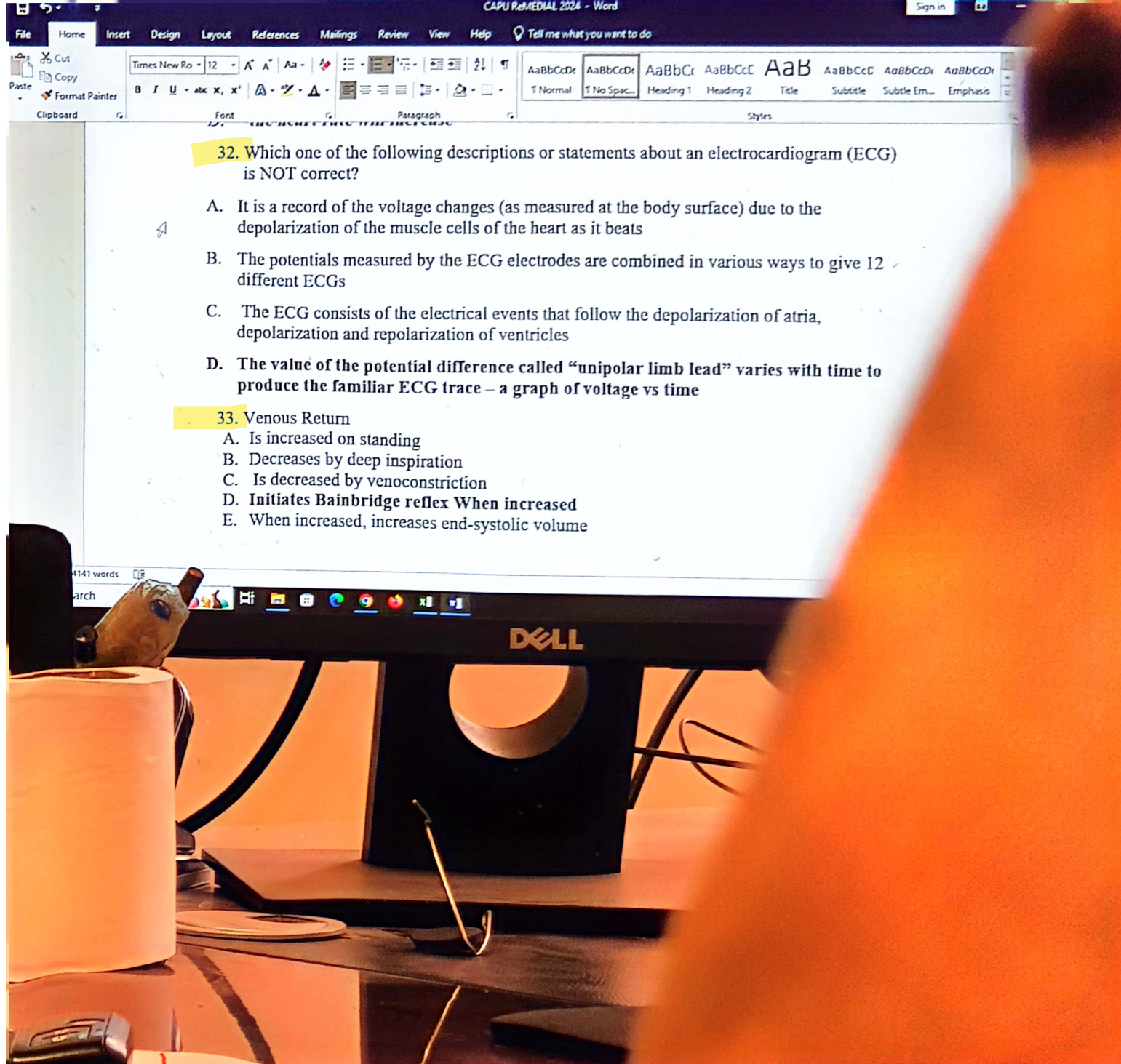
- C. the atria contract, the ventricles are relaxed, the atrioventricular valves are open
- D. the atria are relaxed, the ventricles are starting to relax, the atrioventricular valves are closed, the semilunar valves are closed.

31. What would be a possible consequence of the SA node failing to depolarize?

- A. the entire heart would not contract
- B. the heart rate will decrease B is the correct answer
- C. the ventricles would not contract
- D. the heart rate will increase

32. Which one of the following descriptions or statements about an electrocardiogram (ECG) is NOT correct?

- A. It is a record of the voltage changes (as measured at the body surface) due to the depolarization of the muscle cells of the heart as it beats
- B. The potentials measured by the ECG electrodes are combined in various ways to give 12 different ECGs



32. Which one of the following descriptions or statements about an electrocardiogram (ECG) is NOT correct?

- A. It is a record of the voltage changes (as measured at the body surface) due to the depolarization of the muscle cells of the heart as it beats
- B. The potentials measured by the ECG electrodes are combined in various ways to give 12 different ECGs
- C. The ECG consists of the electrical events that follow the depolarization of atria, depolarization and repolarization of ventricles
- D. The value of the potential difference called "unipolar limb lead" varies with time to produce the familiar ECG trace – a graph of voltage vs time

33. Venous Return

- A. Is increased on standing
- B. Decreases by deep inspiration
- C. Is decreased by venoconstriction
- D. Initiates Bainbridge reflex When increased
- E. When increased, increases end-systolic volume

34. Identify the correct statement about autonomic innervations of the respiratory system

- A. Parasympathetic decreases air way resistance work of breathing
- B. Sympathetic nerve supply arises from sacral segments of spinal cord
- C. Sympathetic activation causes bronchodilation
- D. Autonomic activation controls the smooth muscles of alveoli
- E. Three of the above choices are correct

35. During inspiration all of the following takes place EXCEPT:

- A. Diaphragm and external intercostal muscles contract
- B. The ribs and sternum move up and outward
- C. The abdominal muscles contract
- D. The volume of lung increases but the pressure of the lung decreases
- E. The pleural pressure is far below from the alveolar pressure

36. Identify the correct statement about the pressure and volume system of the normal restful respiratory cycle

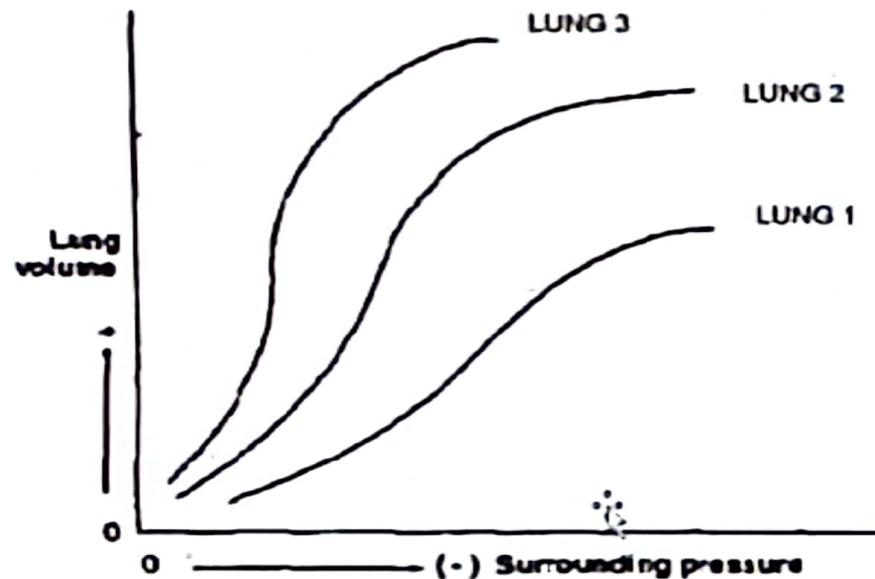
- a. throughout the cycle the alveolar pressure is subatmospheric
- b. ~~Pleural pressure is always sub atmospheric and sub alveolar~~

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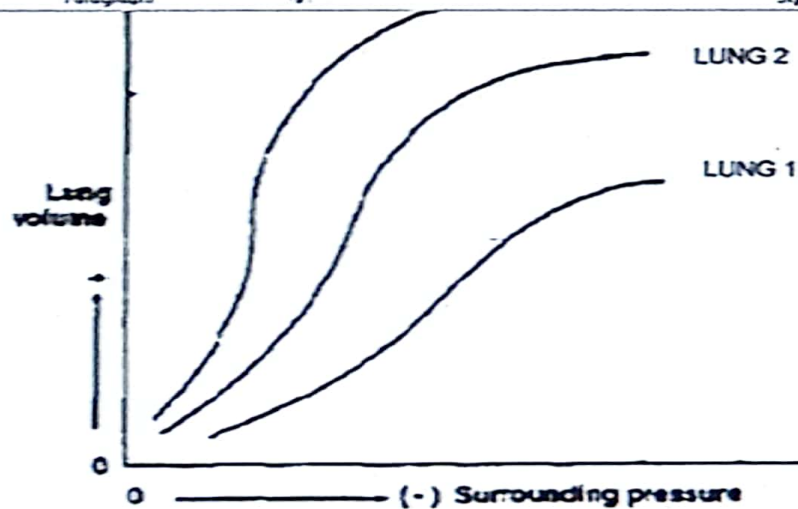
If curve 2 represents a normal lung, which of the following statements is correct?

- A. Lung 1 has obstructive type of respiratory dysfunction
- B. The compliance of lung 3 is less than the compliances of Lung 1 and Lung 2
- C. Lung 3 is more compliant than normal, and at any lung volume recoil will be less than in normal lungs
- D. Lung 1 is more compliant than normal, and at any given lung volume recoil will be greater than in normal lungs
- E. Lung 1 is more compliant than normal, and for a given change in surrounding pressure, more air will flow than in normal lungs



If curve 2 represents a normal lung, which of the following statements is correct?

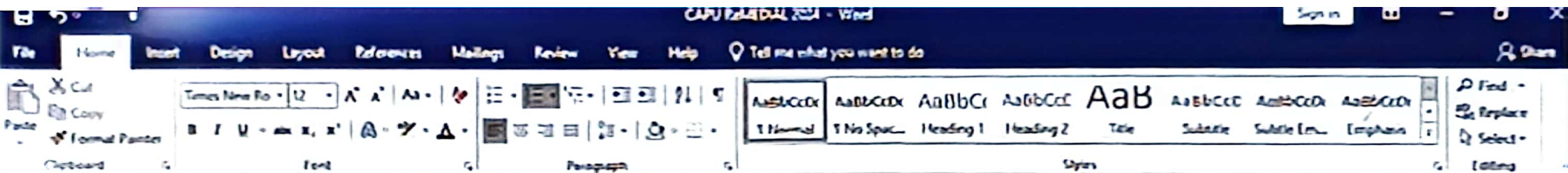
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- E. Lung 1 is more compliant than normal, and for a given change in surrounding pressure, more air will flow than in normal lungs

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38. In asthmatic patients, as compared to normal people which type of work done by the breathing muscles is increased remarkably?
- A. Compliance work
 - B. Tissue resistance work
 - C. Elastic work
 - D. Airways resistance work
 - E. Inelastic work
39. Which of the following pulmonary function cannot be measured directly by spirometry?
- A. Inspiratory reserve volume
 - B. Functional residual capacity
 - C. Expiratory reserve volume
 - D. Tidal volume
 - E. FEV1
40. Gas exchange at the tissue bed and pulmonary bed involves
- A. Active transport mechanism
 - B. Facilitated transport mechanism
 - C. Simple diffusion
 - D. Against partial pressure gradient
 - E. Carrier mediated transport
41. Identify the incorrect statement

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E. Carrier mediated transport

41. Identify the **incorrect** statement

- A. **The bulk of CO₂** transport in the blood is by hemoglobin
- B. Fetal Oxy-hemoglobin curve is more steeper than that of maternal curve
- C. The Bohr effect is important for O₂ transport
- D. Basal O₂ utilization coefficient as blood passes through the tissue is 25%
- E. Face mask has more O₂ delivery than intranasal cannula

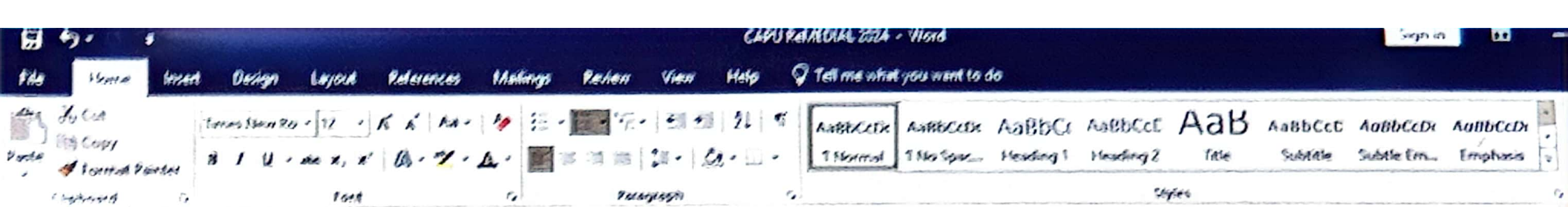
42. Bulk of O₂ transport in the blood involves

- A. **Hemoglobin**
- B. Dissolved form in the plasma
- C. Bicarbonate
- D. Myoglobin
- E. Hemoglobin and dissolved form have equal share

43. Which part of the pontine respiratory center signaling is primarily responsible for increased the frequency of breathing?

- A. **Pneumotaxic center of pontine**
- B. DRG of medullary center
- C. VRG of medullary center
- D. apneustic center of pontine
- E. vasomotor center of medulla

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44. A 70 years old known hypertensive patient suffered a sudden onset of body weakness due to stroke associated with the uncontrolled hypertension. The stroke affected the part of brainstem controlling the rhythm of breathing and the sensory termination from central and peripheral chemoreceptors. This part of the breathing center is

- A. Apneustic center
- B. Pneumotaxic center
- C. Ventral respiratory center
- D. Corpora quadrigemina
- E. **Dorsal respiratory center**

45. Respiratory resuscitation through oxygen therapy is least important for:

- A. Anemic hypoxia
- B. Hypoxic hypoxia
- C. Stagnant hypoxia
- D. **Histotoxic hypoxia**
- E. Hypoxia due to embolism

46. Which of the following would **not** favor the unloading of oxygen from hemoglobin as blood passes through the tissues?

- A. **Increased pH in tissue**
- B. Increased P50
- C. Increased 2, 3-diphosphoglycerate level
- D. Increased level of CO₂

increased 2,3-diphosphoglycerate level.

D. Increased level of CO₂

E. Increased temperature

47. Identify the **wrong** statement about the lung compliance

A. is the ratio of change in lung volume to the change in pressure

B. emphysematous lung has higher compliant than fibrotic lung

C. compliance work is the least dominant work to cause inspiration

D. collapsing pressure is inversely related to alveolar radius

E. The maximum compliance of the lung is at the FRC equivalent

48. The most specific of all biochemical markers for cardiac damage:

a. Troponin I

b. CK-MB

c. Myoglobin

d. Total CK

e. CK-MB mass

49. A 54-year old male was rushed to emergency when he collapsed in the middle of a business meeting. Examination revealed excessive sweating and high blood pressure. Chest ECG was conclusive of Acute Myocardial infarction. Which biochemical investigation out of the following would be of no help in the confirmation of diagnosis?

a. Cardiac Troponins

b. Serum myoglobin

c. Lactate dehydrogenase

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50. Lipoprotein lipase (LPL) is the endothelial cell-associated enzyme necessary for release of fatty acids from circulating lipoproteins. Which of the following apoproteins is required to activate LPL-mediated release of fatty acids from chylomicrons?

- a. apoA
- b. apoB-48
- c. apoB-100
- d. **apoC-II**
- e. apoE

51. Which of the following activities is **most likely** reduced or absent in the presence of the inhibitory compound that directly interferes with the action of prostacyclin (PGI₂)?

- a. Activation of 5-Lipoxygenase for leukotriene synthesis
- b. Enhancement of neutrophil adhesion
- c. Induction of slow cardiac contraction
- d. Platelet aggregation and activation
- e. **Relaxation of vascular smooth muscle cells**

52. Which enzyme estimation will have the best predictive value on the **first day** following onset of acute myocardial infarction?

- a. Serum AST
- b. **Serum CK**

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c. Induction of slow cardiac contraction

d. Platelet aggregation and activation

e. Relaxation of vascular smooth muscle cells

52. Which enzyme estimation will have the best predictive value on the first day following onset of acute myocardial infarction?

- a. Serum AST
- b. Serum CK
- c. Serum ALT
- d. Serum LDH
- e. Serum ALP

53. During postnatal period, cardiac muscle cells primarily uses _____ as an energy source.

- a. Glucose
- b. Amino acids
- c. Ketones
- d. Fatty acids
- e. Lactate

54. Which of the following is not among several events occurring simultaneously in cardiomyocytes during ischemia?

- a. Increased ATP generation from mitochondrial oxidative phosphorylation
- b. Inhibition of cardiac muscle contraction
- c. Increased cytosolic AMP concentration
- d. Increased cytosolic Ca²⁺ concentration

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55. Which of the following is the correct order of specificity of cardiac biomarker for diagnosis of acute myocardial infarction?

- a. cTn-I → CK-MB → Total CK → Mb
- b. CK-MB → Total CK → Mb → cTn-I
- c. Total CK → Mb → cTn-I → CK-MB
- d. Mb → cTn-I → CK-MB → Total CK
- e. Mb = cTn-I → CK-MB → Total CK

56. Which one is not associated with Respiratory Syncytial Virus (RSV) infection?

- A. Most important cause of lower respiratory tract infection
- B. Usually, infants under one year are highly susceptible
- C. Progression of symptoms very rapid, culminating to death

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55. Which of the following is the correct order of specificity of cardiac biomarker for diagnosis of acute myocardial infarction?

- a. cTn-I → CK-MB → Total CK → Mb
- b. CK-MB → Total CK → Mb → cTn-I
- c. Total CK → Mb → cTn-I → CK-MB
- d. Mb → cTn-I → CK-MB → Total CK
- e. Mb = cTn-I → CK-MB → Total CK

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56. Which one is not associated with Respiratory Syncytial Virus (RSV) infection?

- A. Most important cause of lower respiratory tract infection
- B. Usually, infants under one year are highly susceptible
- C. Progression of symptoms very rapid, culminating to death
- D. There is no vaccine available till now for prevention of RSV infection.
- E. Virus is transmitted by infected mother to fetus through placenta

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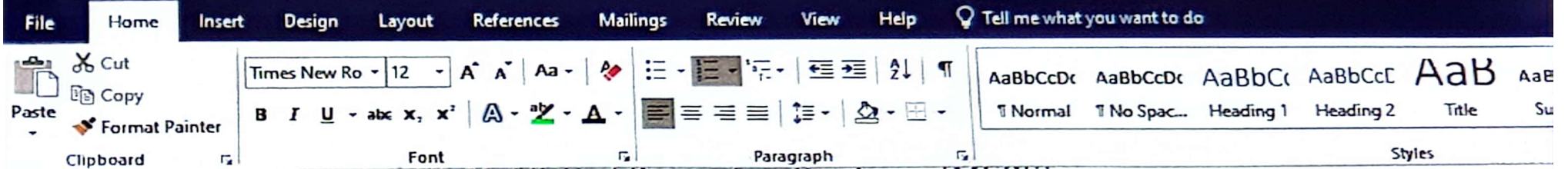
E. Hardy viruses which can be stored at 0–4°C for weeks without loss of viability.

58. Each of the following statements concerning herpes virus latency is correct except

- A. Exogenous stimuli can cause reactivation of latent infection, with induction of symptomatic disease.
- B. Viral persistence in latently infected ganglia lasts for the 2 to 3 years.
- C. Reactivation of latent herpes viruses is more common in patients with 'impaired cell-mediated immunity than in immunocompetent patients.
- D. Virus can be recovered from latently infected cells by co cultivation with susceptible cells.
- E. Only a very few genes are expressed in this stage

59. MiddleEast Respiratory Syndrome (MERS) is

- A. Sexually transmitted



59. Middle East Respiratory Syndrome (MERS) is

A. Sexually transmitted

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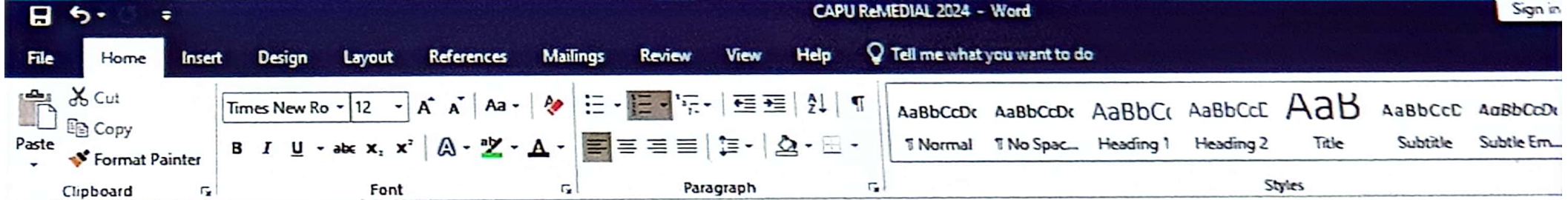
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B. Good vaccine for prevention is cold adapted live

C. Acyclovir is drug of choice

D. Zoonotic infection

E. Caused by a herpes virus type 6



- B. koala
- C. gravis
- D. intermedius
- E. mitis

61. The characteristic of runny nose, sneezing, low fever, and a mild cough is seen in which clinical stage of Pertussis.

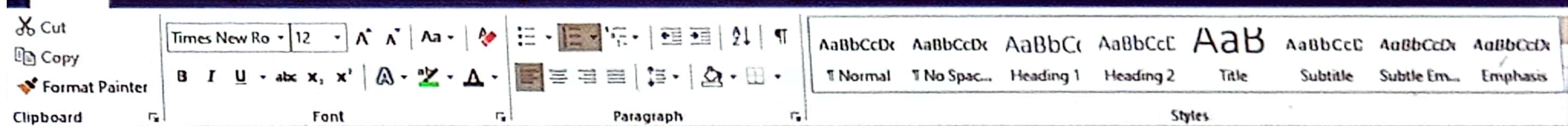
- A. Catarrhal stage
- B. Paroxysmal stage
- C. Convalescence stage
- D. Colonization stage
- E. Final stage

62. *M. pneumoniae* infections have the following features except?

- A. Common cause of CAP in 5- to 20-year-olds
- B. More common during the summer and fall

4167 words

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E. Final stage

62. *M. pneumoniae* infections have the following features except?

- A. Common cause of CAP in 5- to 20-year-olds
- B. More common during the summer and fall
- C. Transmitted from person to person sexually
- D. Resistance for drugs which inhibit cell wall synthesis
- E. Act as a superantigen which stimulates inflammatory cells

63. A red-brown pigment which is produced by *Pseudomonas aeruginosa* called?

- A. Fluorescein
- B. Pyorubin
- C. pyocyanin

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64. The following guideline is mandatory during blood sample collection except?

- A. Avoid hemolysis during collection
- B. Collect the blood before initiation of antibiotic treatment
- C. Collect the blood during the temperature of the patient rises
- D. Collect into bottles with infusion broth

E. Fluid thioglycollate medium is used for aerobic cultivation

65. Which of the following viral pathogens is the main cause of myocarditis?

- A. Influenza viruses
- B. Hepatitis viruses
- C. Coxsackie viruses
- D. Rhinoviruses
- E. Adenoviruses

66. Which of the following is the leading cause of acute bacterial endocarditis?

A. Staphylococcus aureus

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68. Currant jelly sputum is characteristic of pneumonia caused by which bacteria?

- A. *Streptococcus pyogenes*
- B. *Mycoplasma pneumoniae*
- C. *Legionella pneumophila*
- D. *Chlamydia pneumoniae*

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E. *Klebsiella pneumoniae*

69. *Streptococcus pyogenes* is

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4172 words

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E. Motile

e. Streptolysin O

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E. Motile

70. Which one of the following is cellular virulence factor of *Streptococcus pyogenes*

a. Erythrogenic exotoxin

b. Streptokinase

c. C5a Peptidase

d. Hyaluronic acid

e. Streptolysin O

71. Facilitates the degradation of complement system component C3b

a. Protein F

b. lipoteichoic acid

c. M protein

d. Streptolysin O

e. Streptolysin S

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c. M protein

d. Streptolysin O

e. Streptolysin S

72. A 22-year-old cystic fibrosis patient presents because of fever and increasing dyspnea. A gram-negative organism is found in unusually high numbers in the mucus. Which virulence factor is most important in colonization and maintenance of the organism in the lungs?

a. Exotoxin A

b. Pyocyanin (blue-green pigment)

c. Polysaccharide slime

d. Endotoxin

e. Endospore

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- b. By tick exposure
- c. By mosquito exposure

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74. If a culture of the patient's lesion contains many gram positive, catalase-negative, beta-hemolytic, and bacitracin-sensitive cocci, which of the following is the most likely cause of this patient's infection?

- a. *Staphylococcus aureus*
- b. *Streptococcus pyogenes*
- c. *Streptococcus pneumoniae*
- d. *Enterococcus faecalis*
- e. *Neisseria meningitidis*

75. Coronary artery disease: Which of the following coronary artery occlusion has worse prognosis in the right dominant circulation:

- Complete obstruction of the right coronary artery
- Complete obstruction of the distal part of the left anterior descending coronary artery
- Complete obstruction of the proximal part of the left anterior descending coronary artery

e. *Neisseria meningitides*

75. Coronary artery disease: Which of the following coronary artery occlusion has worse prognosis in the right dominant circulation:

- a. Complete obstruction of the right coronary artery
- b. Complete obstruction of the distal part of the left anterior descending coronary artery
- c. Complete obstruction of the proximal part of the left anterior descending coronary artery both a and c
- d. Complete obstruction of the left circumflex coronary artery
- e. Complete obstruction of the diagonal branch of left anterior descending coronary artery

76. The most common bacteria involved in prosthetic valve endocarditis:

- a. Cardiobacterium
- b. Actinobacillus
- c. Streptococcus viridans
- d. Staphylococci epidermidis
- e. Kingella

77. A previously healthy 20-year-old marathon runner suddenly collapses and dies during a training session. His father had suffered a similar fate in his early 30s. On autopsy, he is found to have a hypertrophied heart with an enlarged intraventricular septum. Which of the

e. Complete obstruction of the diagonal branch of left anterior descending coronary artery

76. The most common bacteria involved in prosthetic valve endocarditis:

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77. A previously healthy 20-year-old marathon runner suddenly collapses and dies during a training session. His father had suffered a similar fate in his early 30s. On autopsy, he is found to have a hypertrophied heart with an enlarged intraventricular septum. Which of the following is true?

- a. Histologic examination of the myocardium will reveal no abnormality.
- b. The basic abnormality in the heart is inability to fill left ventricle during diastole.
- c. The most likely cause of death is due to systolic dysfunction leading to heart failure.
- d. The most common cause is Cocksackie group A and B virus.

d. The most common cause is Cocksackie group A and B virus.

78. John's criteria: One of the following is the major criteria to diagnose Rheumatic Fever:

- a. Two separate positive blood culture
- b. Splinter hemorrhage
- c. Prolonged PR on ECG
- d. Migratory polyarthritiis
- e. Fever $>38.5^{\circ}\text{C}$

79. One of the following is commonly associated with variant angina:

- a. Atherosclerosis
- b. Arteriolosclerosis
- c. LDL
- d. HDL
- e. Smoking

80. The hallmark of acute rheumatic carditis:

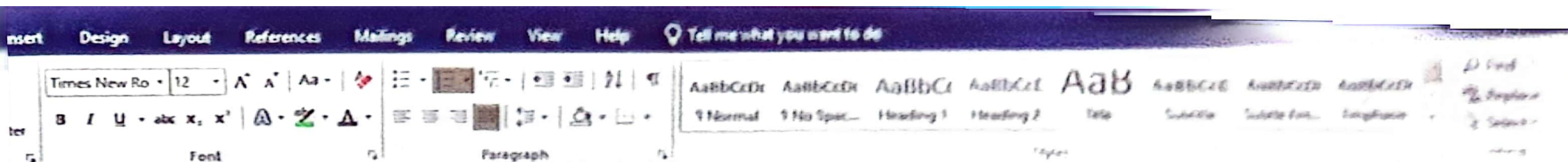
- a. Aschoff bodies
- b. Anitschow cells
- c. Osler's nodules
- d. Aschoff giant cells
- e. Plasma cells

81. Ischemic heart disease: One of the following is not a complication of heart failure:

- a. Pulmonary oedema
- b. Liver necrosis

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e. Plasma cells

81. Ischemic heart disease: One of the following is **not** a complication of heart failure:

- a. Pulmonary oedema
- b. Liver necrosis
- c. Ischemic stroke
- d. Sepsis as a result of intestinal necrosis
- e. **Disseminated intravascular coagulopathy**

82. All are pathologic features seen in hypertrophic cardiomyopathy except:

- a. Left ventricular outflow obstruction
- b. Myocardial fibrosis
- c. Haphazard branching myocytes
- d. Asymmetric septal hypertrophy
- e. **Left ventricular dilatation**

83. A 75-year-old man presents with acute chest pain, shortness of breath and EKG changes concerning for acute myocardial infarction (ST segment elevation). At autopsy, an eccentric arterial wall thickening with thrombus filling the lumen when examining the coronary arteries. Which of the following is a risk factor for this finding?

- a. Coronary artery plaque with thick fibrous cap
- b. **Lipid rich coronary artery plaque with thin fibrous cap**
- c. Low blood triglyceride level

c. Sepsis as a result of intestinal necrosis - d is 2 answer 1 and 2

e. Disseminated intravascular coagulopathy

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- a. Left ventricular outflow obstruction
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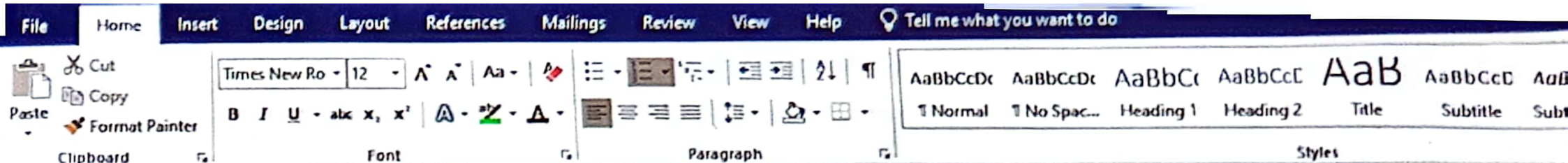
83. A 75-year-old man presents with acute chest pain, shortness of breath and EKG changes concerning for acute myocardial infarction (ST segment elevation). At autopsy, an eccentric arterial wall thickening with thrombus filling the lumen when examining the coronary arteries. Which of the following is a risk factor for this finding?

- a. Coronary artery plaque with thick fibrous cap
- b. Lipid rich coronary artery plaque with thin fibrous cap
- c. Low blood triglyceride level
- d. Patient is a non-smoker
- e. Patient is an athlete

84. One of the following is **not** a feature of left sided heart failure:

- a. Heart failure cells

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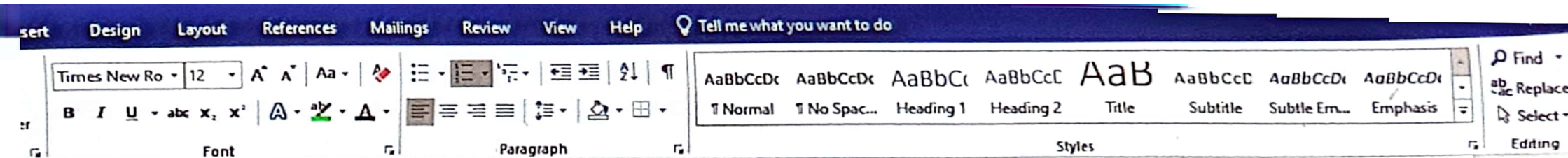


- e. Patient is an athlete
84. One of the following is **not** a feature of left sided heart failure:
- a. Heart failure cells

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- b. Osteopenia
- c. Nutmeg liver
- d. Cyanosis
- e. Dyspnoea

4197 words



85. One of the following are known to present on either or both side of the valve leaflet:

- a. Non-bacterial thrombotic endocarditis
- b. Rheumatic vegetation/Verrucous
- c. Libmann-sack nodules
- d. Infective endocarditis vegetation
- e. None of the above

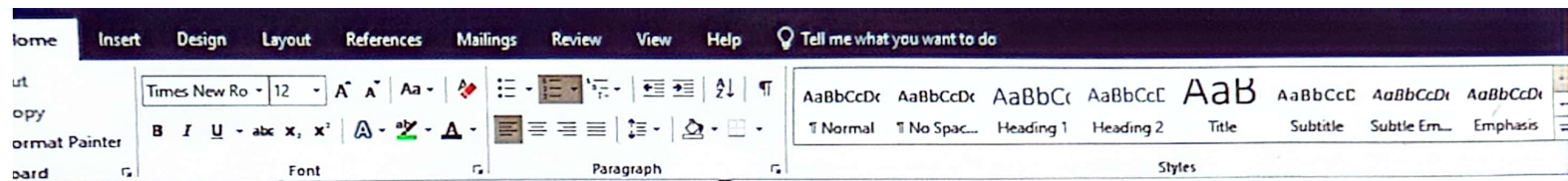
86. Which of the following represents episodic myocardial ischemia caused by coronary artery spasm?

- a. Crescendo angina
- b. Prinzmetal angina
- c. Stable angina
- d. Typical angina
- e. Unstable angina

87. Variant angina is treated with

- a) Nicorandil
- b) Nifedipine
- c) Propranolol

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- e. None of the above
86. Which of the following represents episodic myocardial ischemia caused by coronary artery spasm?
- a. Crescendo angina
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87. Variant angina is treated with
- a) Nicorandil
 - b) Nifedipine
 - c) Propranolol
 - d) Verapamil
 - e) Diltiazem
88. The primary site of action of
- a) Hydrochlorothiazide is descending limb of loop of Henle

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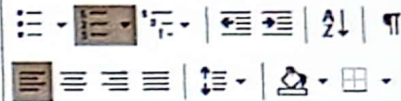
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89. Vasodilators with IV fluid are used in treatment of

a) Drug induced shock

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b) Cardiogenic shock

c) Neurogenic shock

d) Septic shock

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90. A diuretic used for treatment of diabetes insipidus is

- a) Frusemide
- b) Manitol
- c) Acetazolamide
- d) Spiranolactone
- e) Hydrochlorthiazide

91. Antiangina drugs act by

- a) Dilating atherosclerotic coronary arteries
- b) Dilating peripheral blood vessels
- c) Activating beta₁ receptors
- d) Blocking potassium channel

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- a) Frusemide
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- e) Hydrochlorthiazide

91. Antiangina drugs act by

- a) Dilating atherosclerotic coronary arteries
- b) **Dilating peripheral blood vessels**
- c) Activating β_1 receptors
- d) Blocking potassium channel
- e) Opening calcium channel

180/110mmHg. She was in a state of coma. Which drug should this patient be treated with?

- a) Oral captopril
- b) Intravenous nitropruside**
- c) Inhalational propranolol
- d) Oral hydralazine
- e) Oral nifedipine

93. A patient with hepatic edema needs to be treated with

c) Opening calcium channel

92. A 50 years old woman came to an emergency department. Her BP was 180/110mmHg. She was in a state of coma. Which drug should this patient be treated with?

- a) Oral captopril
- b) Intravenous nitropruside
- c) Inhalational propranolol
- d) Oral hydralazine
- e) Oral nifedipine

93. A patient with hepatic edema needs to be treated with

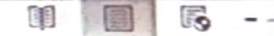
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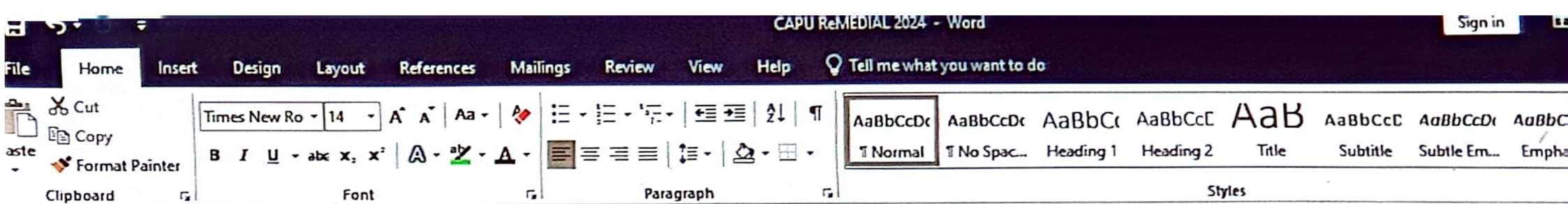
- a) Hydrochlorothiazide + ammonium chloride
- b) Hydrochlorothiazide + amiloride
- c) Captopril + digoxin
- d) **Furosemide + spironolactone**
- e) Mannitol + acetazolamide

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94. Which of the following agents are of great importance in acute attacks of asthma?

- a) Long acting inhalational β_2 selective drugs
- b) Inhalational corticosteroids
- c) Intravenous corticosteroids
- d) **Short-acting inhalational β_2 selective agonists**
- e) Short-acting oral β_2 selective drugs

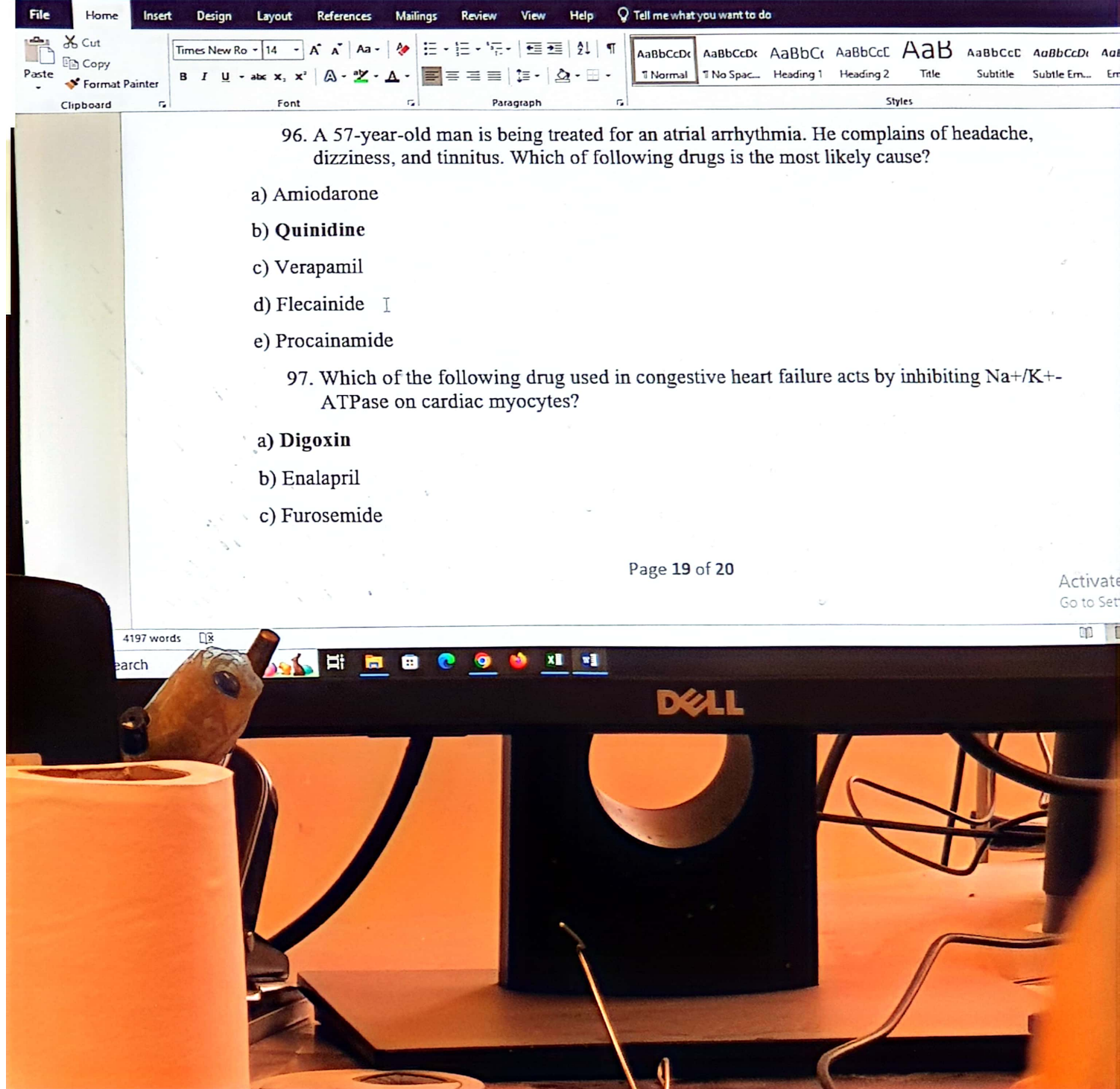
95. Mast cell stabilizers include:

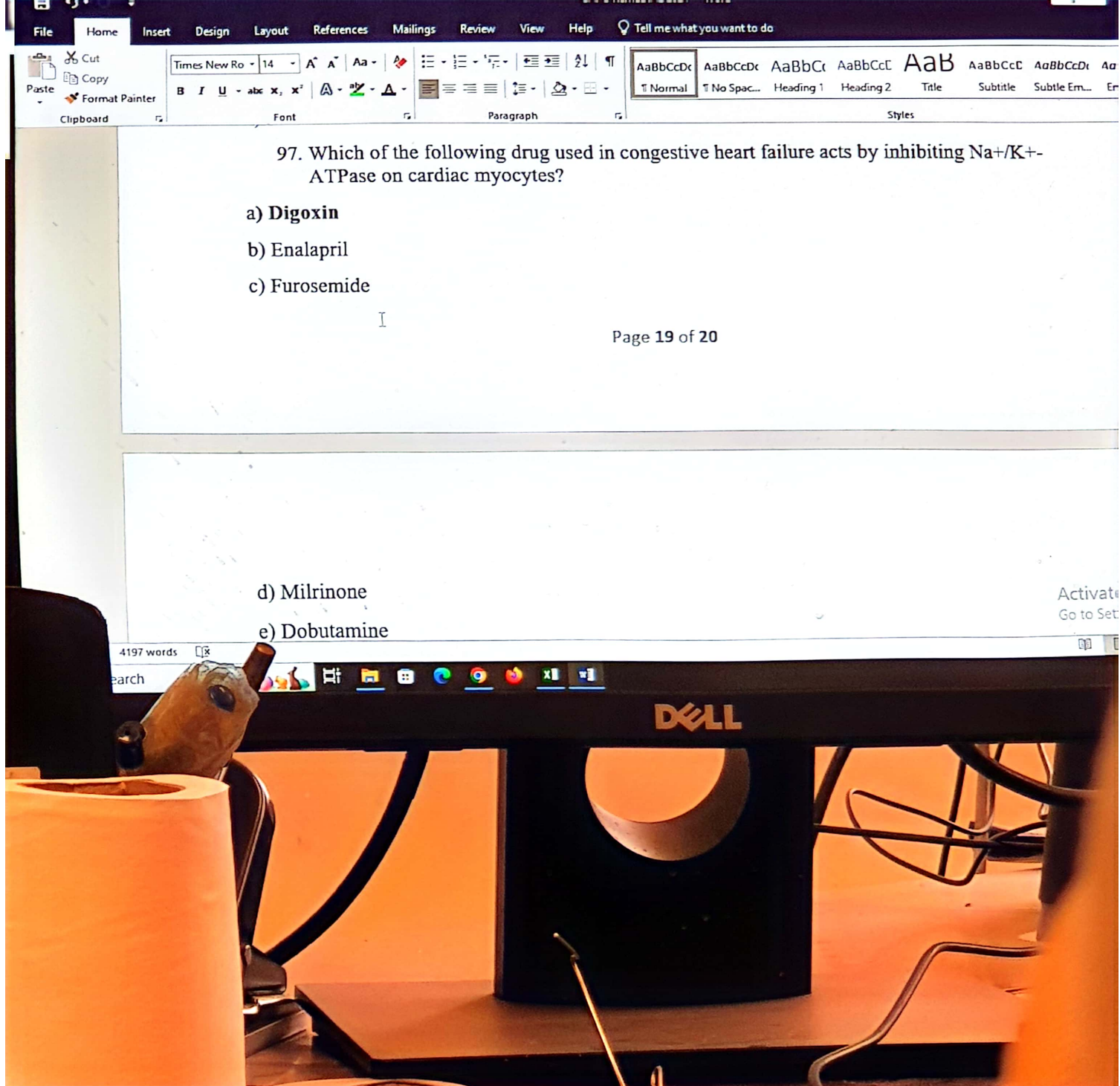
- a) **Nedocromil**
- b) Budesonide
- c) Omalizumab
- d) Montelukast
- e) Aminophylline

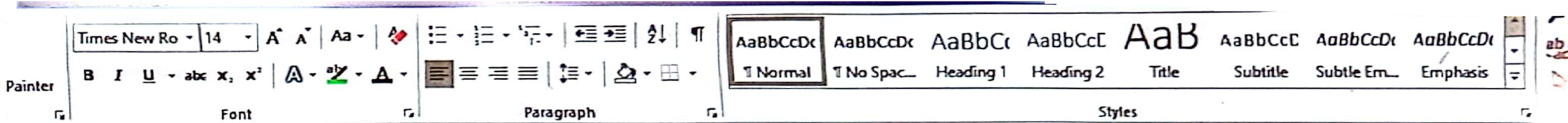
96. A 57-year-old man is being treated for an atrial arrhythmia. He complains of headache.

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98. Antianginal drug that acts by activating ATP sensitive K⁺ channels and acting as a NO donor as well is:

- a) Nitroglycerin
- b) Ranolazine
- c) Nimodipine
- d) Propranolol
- e) **Nicorandil**

99. Which of the following antiarrhythmic agent belongs to sub class I B?

- a) Flecainide
- b) Procainamide
- c) **Lidocaine**
- d) Disopyramide
- e) Quinidine

100. Which of the following statement is correct about cardiac glycosides?

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a) Flecainide
b) Procainamide
c) **Lidocaine**
d) Disopyramide
e) Quinidine I

- a) They are effective in acute decompensated heart failure
- b) Digoxin has longer half-life than digitoxin
- c) Ouabain is more lipid soluble than digoxin
- d) **Digitoxin is eliminated mainly by metabolism**
- e) Cardiac glycosides have wide margin of safety